

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ليلي محمد عبدالغفار احمد

المحافظة : الجيزة

الحي / المدينة : بولاق الدكرور

العنوان : //////////////////////////////////

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 26604182101142 رقم التلفون : 01001363876

العمر(تاريخ الميلاد) : 18-04-1966 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة- الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالم
وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: ليلى محمد عبدالغفار احمد National ID: 26604182101142	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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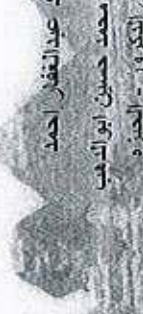
التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

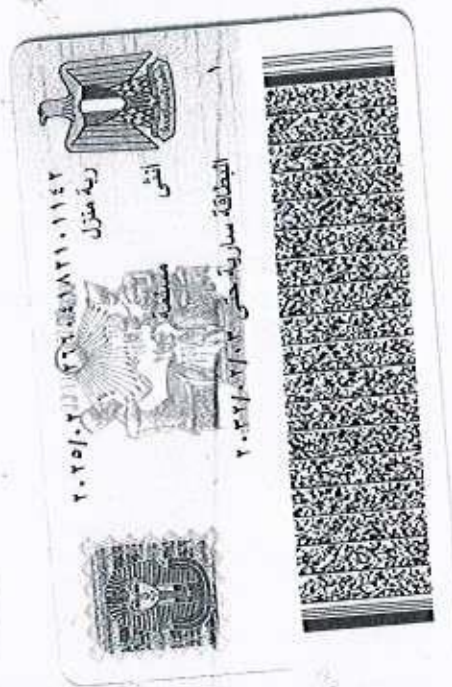
جمهورية مصر العربية
بطاقة تحقيق الشخصية

لبنى
محمد عبدالغفار احمد
ح محمد حسين ابوالغيب
بولاق الشكرو - الجيزة

٢٦٦٠٤١٨٢١٠١١٤٢

LA0578645





Name: <u>ليلى محمد عبد الغفار</u>		
MR. : <u>5565</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 60-year-old individual with a history of hypertension and bilateral breast cancer. The right breast tumor is estrogen-receptor (ER) 8/8, progesterone-receptor (PR) 4/8, HER2-negative, staged clinically as T4N1M1 with de novo bone metastases. The left breast tumor is ER 7/8, PR 4/8, HER2-negative, staged clinically as T4N2M1. Bone scintigraphy performed in May 2024 demonstrated metastatic osseous lesions. A CT of the chest, abdomen, and pelvis in October 2025 showed stable disease, and a repeat bone scan in November 2025 indicated a stationary course. The patient received weekly Taxol combined with Zometa administered every three months, completing nine months of this regimen, with the last Zometa dose given in November 2025. Therapy was then transitioned to letrozole (Femara) together with Zometa every three months beginning in July 2025. Palliative radiotherapy was delivered to the left shoulder in December 2025. The plan at that time was to obtain a new CT CAP to assess treatment response. A CT CAP was performed without a direct comparison; the study was sent for comparison with the prior imaging to evaluate response to Zometa and to assess the elevated CA 12.73 level. Pharmacy records indicate the patient's weight is 112 kg, height 160 cm, and that Zometa and Femara (letrozole) were continued, with a serum calcium of 0.59 mmol/L and CA 12.73 noted. The patient received Femara and supportive therapy today, with a supply intended for two months as ordered by pharmacy. A repeat CT CAP performed in February 2026 demonstrated stable disease. The plan was to continue Femara and supportive care, and to administer Zometa every three months after laboratory results are reviewed, with pharmacy coordination. The most recent instruction was to give Zometa after laboratory results, to add one ampoule of calcium, and to note that the CT from February 2026 showed stable disease, as did the bone scan from November 2025; the plan is to obtain a new CT and bone scan.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	هنا أسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,		

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT Form

A-Patient ID:

1- Name: ليلى محمد عبدالقادر احمد

Sex:

3- Medical number:
Female

83207283

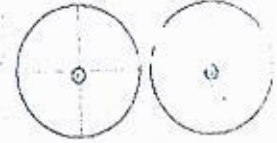
Old Med:

Old Med:

1536-2024

2-ID number: 26604182101142

Patient Age: 58 Y



Family history

Family History

Phone

Phone

01129488183

BMI

BMI

REASON OF MDT

REASON OF MDT
NEW

marital status

Marital Status

NRS Score

NRS Score

4.0/UP

GROUP
N

Date of MDT

Date of MDT

09/06/2024

Co morbidity

Co Morbidity
htnMenopausal
statusMenopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging:

Mets Workup:

MBC to bone

BX
IDCBX COMMENT:
li

ER

ER COMMENT:
rt 8/8
LT 7/8

PR

PR COMMENT:
4/8
4/8

Her 2 Neu

Her 2 Neu COMMENT:
AVE
AVE

SISH

SISH COMMENT:

Ki67

25
30

T

T
T
T

T

1
2

M

1
1

Sonomammography

PATIENT HISTORY

c/wide spread osseous lesion

bone scan 25/5/2024: abnormal metastatic osseous lesion involving skull both scapula, DV12,

pelvic, trachea of both femur, L4

Wide spread bone mets

O/E it fungating MASS

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

* Taxol weekly

* Zometa / 3 ms

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL

/ رشوي صبر

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

PATIENT NAME: LAYLA MOHAMMED ABDULGHAFFAR AHMED
GENDER: Female
DATE : 25-May-2024 01:50 PM

PATIENT ID: 19124242681
AGE: 58 y
ORDER: 2860557

99MTC-MDP BONE SCAN

Desired Information:
Skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Active osseous lesions are seen involving the skull, sternum, both scapulae, DV12, few pelvic bones, trochanter of both femora and likely LV4. Degenerative changes are seen over dorso-lumbar vertebrae. Arthritic changes are seen over both knees. Both kidneys are normally visualized.

Conclusion:

Abnormal metastatic osseous deposits at the above mentioned sites.

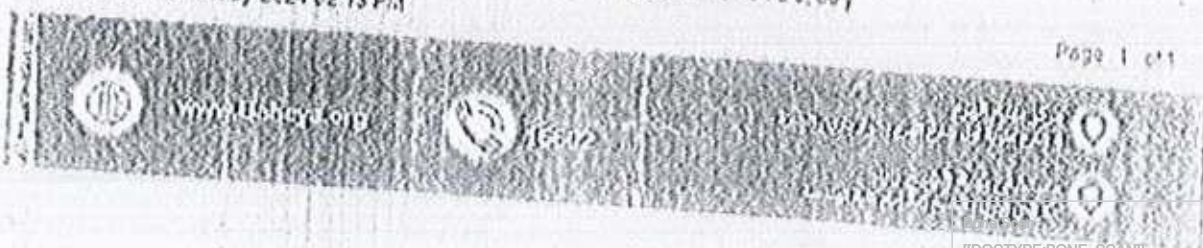
Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

26-May-2024 02:13 PM

PATIENT: LAYLA MOHAMMED ABDULGHAFFAR AHMED, 19124242681, Female, 58 y
Report version 1, 26-May-2024 02:13 PM

Page 1 of 1



Patient Name: Laila Mohamed Abdel Ghaffar Ahmed	Date : 26/04/2025
Exam : C.T CHEST	Age : 59 Year(s)
Medical ID: 1536/2024	Pat. ID: 53907

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Reported case of bilateral breast cancer.

I. Chest

As compared to the last chest study dated 22/02/2025

- **Stationary course:**
 1. Of Left breast multiple irregular spiculated soft tissue lesions, the largest seen retro-areolar measuring 5.8X5 cm I
 2. regarding the size and number of the left axillary level I and, II pathologically enlarged lymph nodes the largest at group I measures 3.1X3 cm.
 3. of the subcentimetric right axillary level I lymph nodes.
 4. of the small left Internal mammary lymph node.
 5. Bilateral diffuse mosaic attenuation with scattered atelectatic bands
- No sizable pulmonary nodules, masses or cavitary lesions
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II. Abdomen and pelvis

As compared to the last chest study dated 09/9/2024.

- The liver is enlarged homogenous CT texture. The previously mentioned hepatic dome hypodense focal lesion measuring 1 cm is not seen currently on non-enhanced CT basis.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Left cortical cyst 23 x 16 mm
- Bulky left adrenal gland with small hypo to isodense lesion 12 x 10 mm
- Right suprarenal gland shows hypodense lesion measuring 11X8mm.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD seen in place.
- Normal appearance of the other pelvic organs.



Patient Name: Laila Mohamed Abdel Ghaffar Ahmed	Date : 23/12/2024
Exam : C.T CHEST	Age : 58 Year(s)
Medical ID: 1536/2024	Pat. ID: 53907

- Still noted Bulky left adrenal gland with small hypo to isodense lesion 12 x 10 mm.
 - Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - IUD seen in place.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
3. The scanned skeleton:

More sclerosis of some of the widespread axial and appendicular skeleton sclerotic and lytic lesions.

OPINION:

- Regressive course of the left breast spiculated mass lesion and left axillary lymph nodes as described
- More sclerosis of some of the osseous metastatic deposits suggesting favorable therapeutic response.

Dr. Fatima Ahmed

Dr. Mohamed Mohsen, M.D.

Patient Name : الأمانة / السيد محمد عبد الحليم
Patient No. : 26604182101142
Path No. : 1660/24
Sex : Female
Age : 58 Y
Referring Doc :

Request No. : 13000289
Request Date : 22/05/2024
Verification Date : 30/05/2024
Financial Category : فئة خاصة
Department : Out Patient
Registered by : Mrs. Mayada Hagazy

Pathology Report

Clinical Data

Left breast cancer with right axillary lymph node

Nature of specimen

Tru cut biopsy, collected at 22/5/2024, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for Immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma, No nodal tissue
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angiolymphovascular Invasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPI (Positive control showed appropriate reactivity):
- * E-cadherin: Positive
- * GATA3: Positive
- * ER: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
- * PR: Weak nuclear reaction in 10 % tumor cells (Allred score 4/8)
- * HER2: No membrane reaction in of tumor cells (score 0)
- * Ki 67: 25 % nuclear reaction of tumor cells

Verified by :

Dr. Heba Hasan

Printing Date : 01/06/2024 09:50:36

Page 1 of 2

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Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / ليلي محمد عبد الغفار
Patient No. : 26604182101142
Path No. : 1680/24
Sex : Female
Age : 58 Y
Referring Doc :

Request No. : 13000299
Request Date : 22/05/2024
Verification Date : 30/05/2024
Financial Category : برنامج دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data

Left breast cancer with right axillary lymph node

Nature of specimen

Tru cut biopsy, collected at 22/5/2024, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench

Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma, No nodal tissue
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
- * DCIS: No
- * Angiolymphatic invasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity):
- * E-cadherin: Positive
- * GATA3: Positive
- * ER: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
- * PR: Weak nuclear reaction in 10 % tumor cells (Allred score 4/8)
- * HER2: No membrane reaction in of tumor cells (score 0)
- * Ki 67: 25 % nuclear reaction of tumor cells

Verified by:

Dr. Hiba Hasan

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Page 1 of 2

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Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mbt El Qadima - Cairo, Egypt

Patient Name : الأستلا / ايلي محمد الطاهر
Patient No. : 26004182101142
Path No. : 1680/24
Sex : Female
Age : 58 Y
Referring Doc :

Request No. : 13000299
Request Date : 22/05/2024
Verification Date : 30/05/2024
Financial Category : فئة الدولة - رواتب ومرتبات
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Diagnosis

Right axillary mass, Tru cut biopsy.

Metastatic Invasive duct carcinoma, Grade II, No nodal tissue in cores

IP1:

- * ER: Positive (score 8/8).
- * PR: Positive (score 4/8).
- * HER2: Negative (score 0).
- * Ki67: Positive in 25% of tumor cells.

Prof. Dr. Lubna Omar
Dr. Mienna Nabil

Signature
Dr. Rana Fathy
Dr. Hiba Hassan

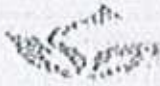
Verified by :
Dr. Hiba Hasan

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Page 2 of 2

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Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / أمال محمد عبد القادر
Patient No. : 26604182101142
Path No. : 1848/24
Sex : Female
Age : 58 Y
Referring Doc :

Request No. : 13009660
Request Date : 20/05/2024
Verification Date : 30/05/2024
Financial Category : فئة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Diagnosis

Left breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II

IPt:

- * ER: Positive (score 7/8).
- * PR: Positive (score 4/8).
- * HER2: Negative (score 0).
- * Ki67: Positive in 30% of tumor cells.

Signature

Dr. Rana Fathy
Dr. Heba Hassan

Prof. Dr. Amal Hareedy

Verified by:

Dr. Heba Hasan

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Handwritten signature in blue ink, likely reading 'Rabea Matar'.

